



Fiona Stanley Fremantle Hospitals Group

Policy and Procedure

Visiting in mental health inpatient areas

Reference #: FSFH-MENH-POL-0016

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital Fremantle Hospital	Mental Health Service	Medical, Nursing, Allied Health and Patient Information Management staff (PIMS)

1. Introduction

Maintaining contact with family, friends and carers is very important during an inpatient stay and it is crucial that safe and efficient processes are established to maximise contact between patients and the significant people in their lives to support recovery. This document stipulates the requirements and processes for managing visitors to the Mental Health Service.

2. Terminology

Visitor	Any person, being a relative, carer, friend, professional, religious, support, legal person who visits the patient in hospital in accordance with the patient's wishes.
Child	Anyone under 18 years of age.
Unsafe items	May include sharp instruments, knives, razors, glassware, pins, spray deodorants, hair ties, plastic bags, matches, lighters, belts, ties, shoelaces, electrical cables/cords, gym equipment, alcohol or other drugs or other items that may be used by a patient to harm themselves or others.

3. Policy

3.1. Patient rights

- Patients will be provided with both verbal and written information on their rights upon admission.

- Identified significant Aboriginal family/community, which may include traditional healers, are strongly supported to be involved in Aboriginal patients mental health care.

Under the Mental Health Act (MHA) 2014 patients have a right to private, lawful and uncensored communication, such as being able to see and speak to others in the hospital and being visited or contacted by others including a legal representative.

- Visitors will be advised that use of the camera or video facilities on their mobile phone is strictly prohibited to ensure patient privacy is maintained.
- Any breach will result in the visitor being asked to delete the image/s and place their mobile phone in the lockers provided for the duration of the visit.

3.2. Environment

- Visiting times will be clearly displayed at the entrance to the hospitals and wards. Alternative arrangements outside visiting hours may be agreed with the treating teams/Nurse Unit Manager (NUM) in extenuating circumstances.
- All visitors to the Mental Health Service must sign the visitors' logbook on entry to the service.

3.3. Safety

- Visitors will remain in common areas and are not allowed to enter the patient's bedroom. Exceptions may apply to the Mother Baby Unit and Older Adult ward and will be documented in the patient's medical record by the treating team.
- Staff will check items given to patients from visitors for unsafe items or other hazards and record any property/money received by a patient in line with the SMHS Searches in Mental Health policy.
- Visitors who are visibly intoxicated on arrival will not be able to enter the Mental Health Service and will be asked to return when they are no longer visibly intoxicated.
- As per the stipulations of the SMHS Workplace aggression and violence policy visitors must treat all staff with respect in the workplace.
- Threatening, intimidating, aggressive or abusive behaviours will not be tolerated. If these requirements are not observed:
 - the visitor/s may be denied access or be asked to leave
 - Security services may be requested to attend
 - the WA Police may be requested to attend

- Routine family and domestic violence (FDV) screening is recommended on initial contact with the Mental Health Service or at any time a clinician suspects a patient may be at risk of FDV or observes possible indicators.

Any person who has a known Family Violence Restraining Order (FVRO) against them will not be permitted to visit the person who is protected by the FVRO as this constitutes a breach of the FVRO conditions.

3.4. Visiting children

- The welfare of a child/children is paramount, therefore visits with a patient's child should only take place after a multidisciplinary discussion to:
 - ascertain the appropriateness of the contact,
 - identify any concerns e.g. identified FDV
 - identify if there are any current Court Orders with conditions such as supervised access
 - assess any risk of harm to the child/children and
 - obtain agreement that it is in the best interests of the child/children.
- The discussion and outcomes will be documented in the patient's medical record and all proposed visits organised with nursing staff prior to the visit.
- Family oriented supports and resources will be put into place for a parent to maintain contact with their children whilst on an inpatient ward e.g. COPMI, Emerging Minds
- In general, children under the age of 16 years will not be permitted on the inpatient wards:
 - alternative locations e.g. 'quiet area', 'family room' or private courtyard area which allow privacy and are suitable for family visiting away from the ward are provided by the Mental Health Service
 - exceptions include the Mother Baby Unit and the Older Adult ward where visiting arrangements may be considered on an individual basis.
- Children must be accompanied by an adult other than the patient or a staff member and remain the responsibility of the accompanying adult throughout the visit.
- In situations where older children (aged between 16 and 18 years) wish to visit independently the request will be individually assessed to determine the safety and appropriateness of an unaccompanied visit.

3.5. General visiting considerations

- Set visiting hours are provided to allow for: mealtimes, clinical reviews and therapeutic programs/groups
- Visitors will be advised to visit patients during ward visiting times. Staff may terminate a visit if the patient's mental state deteriorates or it is suspected the visitor has provided unsafe items to the patient.
- Any need to restrict visiting for clinical reasons will be clearly and empathetically explained to the visitor and the patient.
- Restrictions to visiting will be documented on a Form 12C Restriction of freedom of communication. Refer to the FSFHG "Restriction of freedom of communication" policy and procedure.
- Restrictions to visiting will be clearly documented in the patient's medical record, iSOFT handover and details provided in clinical handover
- Patients' individual visiting arrangements will be documented in their Treatment Support and Discharge Plan (TSDP).

3.6. Infection prevention and control

- Visitors will be asked not to visit if they are symptomatic with respiratory illness, have prior extensive exposure to infectious individuals or if it is thought they impose an infection risk to others.
- Visitors will be asked to wash their hands/use alcohol-based hand rub on entering and leaving the inpatient wards.
- For patients with contact or droplet precautions, Nursing staff will consult with Infection Prevention and Management staff to ascertain whether visiting restrictions will be required

4.Procedure

4.1. FSH Mental Health wards visitor management: General principles

- Visitors present to the main reception and inform the reception staff who they are visiting.
- Reception staff check the list maintained in Reception to determine whether there are any restrictions preventing the individual from visiting.
- Reception staff telephone the relevant ward to advise Nursing staff of the visitor's presence in Reception and the patient they wish to see.
- Prior to speaking with the visitor, the Nurse will consider any potential risks and request Security presence if required.
- If the visitor is restricted, the Nurse explains the restriction to the visitor.

- The Nurse proceeds to Reception to collect the visitor
- Visitors are asked to sign the Visitor Book on entry and when leaving the MHU.
- To avoid harmful/unsafe items being taken onto the wards, visitors are asked to place their personal items in the lockers provided.
 - The visitor retains the key to the locker during the visit. At the end of the visit the key is returned to the escorting Nurse.

4.2. Mother Baby Unit (MBU): Specific requirements

- Partners may board overnight with the patient in preparation for discharge provided prior agreement between the patient and treating team has been reached.
- Where there is no FVRO in place, but family and domestic violence issues are identified on screening or by the patient, admission to the MBU will be on a case-by-case basis in consultation with the patient and the treating team.
- Where there is a FVRO in place there are to be no staff supervised contact visits between a baby and the person bound by the Order.

4.3. FH Mental Health wards visitor management: General principles

- Visitors present to the Triage Reception on Level 4 and advise Reception whom they are visiting.
- Reception staff check the list maintained in Reception to determine whether there are restrictions in place preventing the individual from visiting.
- If restrictions are in place, Nursing staff or the treating team will advise visitors regarding the restrictions.
- If there are no restrictions in place, Reception staff direct the visitor to the relevant ward.
- Families are encouraged to use the Family Room on Level 5 when children are visiting

4.4. Ward W4.1 (secure ward): Specific requirements

- Nursing staff meet the visitor at the Ward W4.1 entrance
- To avoid harmful objects/unsafe items being taken onto the wards, visitors are asked to place their personal items in the lockers provided.
 - The visitor retains the key to the locker during the visit. At the end of the visit the key is returned to the escorting Nurse.
- Visitors with children are directed by Nursing staff to enter the ward via side entrance on Alma Street

- Visits with children occur in the smaller courtyard with no other patients present to ensure privacy and safety.
 - The Treating Team determines the level of supervision to be maintained during the visit following Mental State Examination and risk assessment.

5.Compliance/Performance Monitoring

Behaviour incidents will be reported in Datix Clinical Incident Management System (CIMS) and will be monitored by the ward Nurse Unit Manager /Clinical Nurse Specialist.

6.Related Policy Documents

DOH:

- [Guidelines for Protecting Children 2015 \(revised 2017\)](#)

SMHS:

- [SMHS Searches in mental health](#)
- [SMHS Workplace aggression and violence](#)

FSFHG

- Restriction of freedom of communication (FSFH-MENH-POL-0004)
- Clinical Handover (FSFH-HW-POL-0035)
- Clinical Documentation (FSFH-HW-POL-0039)
- Courtyard access and observation – Mental Health (FSFH-MENH-POL-0005)
- Family and Domestic Violence (FSH-HW-PRO-0007)

7.Related Documents

[The Mental Health Act 2014](#)

[Surveillance Devices Act 1998](#)

8.Related Standards

NSQHS Standards:

- Clinical Governance
 - Addressing the specific health needs of Aboriginal and Torres Strait Islander people
- Partnering with consumers
- Communicating for safety.

NSMHS Standards:

Printed or personally saved electronic copies of this policy are considered uncontrolled. Refer to the FSFHG Policy hub for current controlled electronic policies.

- Rights and Responsibilities
- Safety
- Delivery of Care

Chief Psychiatrists Standards for Clinical Care

9. Authorisation

EXECUTIVE SPONSOR: Service Director, Service 5					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	11/2019	Clinical Nurse Specialist, MBU	NMPGC	FSFHG Policy Committee	11/2022
2	11/2022	Clinical Nurse Specialist, clinical Improvement	NMPGC	Service 5 Safety Quality and Risk Committee	01/2026